

DISCHARGE NOTE

Date of Discharge: [NOT DOCUMENTED]

Time of Discharge: [NOT DOCUMENTED]

Discharging Doctor: Dr. [unclear]

PATIENT DETAILS:

Name: [NOT DOCUMENTED]

Age/Gender: 58 years / Male

IP Number: [NOT DOCUMENTED]

DIAGNOSIS AT DISCHARGE:

1. Hypertensive Urgency
(NOTE: Admission note states Hypertensive Emergency - CONFLICTING DIAGNOSIS - requires clinician clarification)
2. Type 2 Diabetes Mellitus - Uncontrolled
3. Hypertensive Nephropathy vs Pyelonephritis
(CONFLICTING IMPRESSION - nephrology opinion pending)
4. Left Ventricular Hypertrophy (ECG finding)

CONDITION AT DISCHARGE:

Hemodynamically stable. BP 150/90 mmHg at discharge.

Headache resolved. Vision improved.

ADVICE ON DISCHARGE:

Medications:

1. TAB. AMLODIPINE - 10MG - OD - Continue
2. TAB. TELMISARTAN - 40MG - OD - 30 days
3. TAB. FUROSEMIDE - 40MG - OD - 7 days
4. TAB. METFORMIN - 500MG - BD - Continue
5. TAB. ASPIRIN - 75MG - OD - 30 days

(NOTE: NEW MEDICATION - NOT IN DRUG CHART - NO DOCUMENTED REASON)

FOLLOW UP INSTRUCTIONS:

- Review after 1 week with repeat RFT, CBC, Urine Routine
- BP monitoring daily at home
- Low salt, low fat diet
- Diabetologist opinion for uncontrolled T2DM
- Nephrology follow up for renal impairment

PENDING RESULTS:

- Renal Function Test (RFT) complete panel - AWAITED
- Nephrology opinion - AWAITED

DIET ADVICE:

- Low salt diet (less than 2gm sodium per day)
- Low fat diet
- Diabetic diet - avoid simple sugars

Discharge Summary prepared by: [unclear]

Consultant Signature: [unclear]

THIS DISCHARGE IS AT PATIENT'S REQUEST AFTER CLINICAL STABILIZATION. PATIENT COUNSELLED REGARDING RISKS.